

- Rock gently for momentum and count to three. In one continuous movement, stand, pivot, (Figure 11), shuffle together backward until the chair touches the back of the knees, and lower the person into the chair. (See figures 14 and 15)
- Replace wheelchair armrest and foot pedal.

WALKING

Although the person may be able to stand and walk, help may still be needed to prevent falls. Some people will be able to use a cane or walking device while others will need support.

How you can offer care

- The most important thing for you to do is prevent your loved one from falling while walking.
- Move all obstacles from your path. Have a chair nearby in case the person needs a rest.
- Make sure both of you are wearing non-skid footwear.
- Provide your support on the person's weaker side. If a cane is used, have the person hold it on the stronger side. This will keep the weight on the side that can support it.
- Stand beside and slightly behind the person, facing the same direction. When necessary, remind the person to stand tall and to look ahead, not down at the floor.
- Put your arm around the person's waist and use your other hand to hold the person's elbow or hand. Stay close so that your entire body gives support. (See figure 16)
- Try using a belt or folded blanket around the person's waist so you can hang on to it to give added support.

7. Toileting

For some people, the need for help with toileting can be very embarrassing. This is particularly true for someone who is confined to bed.

What you need to know

- The person may need help to use the toilet, a commode, a urinal or a bedpan depending on mobility.
- When helping with toileting needs, respect patient's dignity.
- Be sensitive to the need for privacy.

- Be matter-of-fact about the activity to reduce embarrassment.

Fig 17.

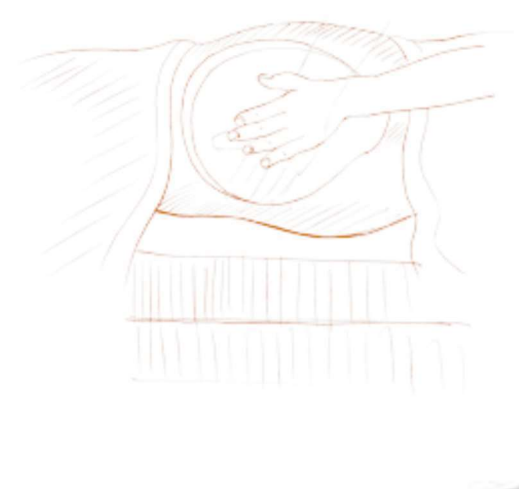


Fig 18.



Fig 19.



Bathroom

When the person is able to get up and go to the toilet, offer whatever help is needed.

Stay close by.

- Make sure the bathroom floor is dry, the person has non-slip footwear and the path to the toilet is well-lit and free of obstacles.
- Place toilet paper within reach.
- Give privacy if the person can be left alone.

- Allow as much time as needed. The sound of running water might help someone who is having trouble urinating.
- Help with wiping-up as needed. Wipe from front to back.
- Help the person to wash when finished, then wash your own hands.
- Take your time getting back to the bed or chair.

Commodes

Commodes are portable toilets in the shape of a chair. They can be positioned close to the bed of a person who is able to get up but too weak to walk to a bathroom.

- Be sure the brakes are locked on the commode.
- Use the techniques described in Moving from Bed to Chair to help the person move to, and use the commode.
- Let the person do as much as possible. Have toilet paper within reach.
- Help the person to wash hands.
- Empty the commode bucket as soon as you have helped the person back to the bed or chair.
- Wash your hands and return the commode to its usual place.

Urinals

Urinals are small collection bottles that men can use for passing urine. They come in different shapes and sizes and are usually made of plastic. Some are made of metal or molded cardboard.

- Some men are able to use the urinal lying down, while others prefer sitting on the edge of the bed or standing up. If the person wants to stand, offer support.
- If the person uses the urinal in bed, raise the head of the bed for comfort. Make sure the foot of the bed is down so that urine does not spill out.
- Keep the urinal emptied and thoroughly rinsed after every use. This will help to prevent spills and offensive odors. If you rinse with cold water and baking soda, it will keep odor down.
- Wash your hands after emptying the urinal.
- If the person wants help using the urinal, be sure the penis is placed directly into the urinal and that the urinal is tilted downward.

Bedpans

Most people find a bedpan uncomfortable and awkward to use but if a person is not able to get out of bed, it may be necessary.

- Warm up a bedpan if you want to by rinsing it with hot water and then drying.
- Try some talcum powder on top of the bedpan so it does not stick to the skin.
- Place the bedpan by asking a person who is strong enough to lie with knees bent and feet flat on the mattress. Help lift the person's bottom while you slide the bedpan under.
- Alternatively, help the person roll away from you. Place an incontinence pad on the mattress, put the bedpan on the mattress where the person's bottom will be and help the person roll back onto the pan. (See figures 17, 18 and 19)
- Raise the head of the bed to increase comfort. Have the foot of the bed down so that urine will not pour out by mistake.
- Make sure the person is wiped clean and dry.
- Cover the bedpan before removing it to prevent spilling. Empty it in the toilet and clean. Rinse with cold water and baking soda to keep it odor free.
- Wash your hands and help the person to wash.

8. Food & Fluid Needs

DECREASING APPETITE

What you can expect

Enjoying food and drink together is a part of our culture. It is difficult to watch someone you love eat less than usual.

- A decreasing appetite is normal because the body cannot digest food as it once did.
- The person may refuse solids and will only drink liquids.
- There may be a noticeable weight loss. This may happen because of the disease; no matter how much is eaten.
- A changing sense of taste may alter the enjoyment of food. Sometimes this is only temporary.
- Bitter tastes may develop or sometimes food may seem too sweet.
- Some people develop distaste for meat or for certain textures of foods and certain smells.

How you can offer comfort and care

Try to find ways to tempt the person's appetite and give nourishing foods but remember that there is no harm going without food sometimes. The important thing is that the person continues to drink fluids.

- Try new spices and flavorings for foods. It is common for a person's preferences to change during illness.
- Avoid highly seasoned or salty foods.
- Add sauces and gravies to dry food.
- Flavor food with sugar, basil, seasonings, lemon juice or mint.
- Add fruit and juice to milkshakes, custards, ice cream and puddings.
- Marinate meat in soya sauce, sweet juices or sweet wines.
- Try alternative high protein foods such as eggs, poultry or fish for someone who has developed a dislike of meat.
- Give high protein, high calorie snacks such as egg-nogs, cream soups and ice-cream.
- Choose foods that are soft and easy to eat.
- Avoid foods that have similar textures to the foods the person dislikes.
- Try serving water, tea or soft drinks to take a strange taste away. Sometimes citrus juices such as lemon added to foods can make a taste more normal.

GIVING HELP WITH EATING

The act of eating can cause problems. As well as having no appetite the person may not have the energy to eat.

How you can offer comfort and care

Even though your patient may be having difficulty eating, there are some things you can do to make it easier.

- Encourage the person to rest after meals, but keep the head of the bed elevated to help digestion.
- Make adjustments to the diet if the person can no longer wear dentures. Soft foods or small bite-size portions of meat, softened with gravy are ideal.
- Remember that the person may forget to eat. Offer small snacks throughout the day rather than waiting to be asked.

- Assess the person's ability to chew and swallow before you serve solid foods. For someone who can swallow but cannot chew, a puree or pudding would be most effective. For someone who can chew, keep food pieces small so less energy is needed to eat.
- Give finger foods if the person prefers or can only eat with fingers. This will help to maintain a level of independence.
- Be sure that the person's head is well supported and upright when eating or being fed.
- Use bibs or large napkins when necessary to help keep clothing and bedding clean.
- Use a spoon instead of a fork when you are feeding someone. This will prevent accidental stabs with a fork prong. Also, a long-handled spoon will help you place the food far enough into the mouth.
- Offer small spoonful's and place food at the front of the mouth. Wait until the last spoonful is swallowed before offering the next.
- Give liquids and solids separately.
- Practice feeding with a friend or family member. Switch roles so you are aware of both sides of the feeding experience.
- Keep a small kidney basin or bowl close by. Nausea and vomiting can happen very suddenly so be prepared.

PREVENTING DEHYDRATION

Fluids help to flush out body waste products and keep cells and skin healthy. If your loved one is no longer able to drink enough fluids, dehydration may occur, meaning there is not enough fluid in the body tissues. Dehydration can cause weakness, nausea, confusion and restlessness.

What you need to know

- One of the first indications of dehydration is less urine being produced.
- In dehydration, the color of the urine will be very dark yellow or tea-colored with a strong odor.
- Wastes will not be removed from the body and the person may become confused.

How you can offer comfort and care

- Always have fluids close at hand. These include water, juice, coffee, tea, ice chips, broths and nutritional supplements. Water flavored with lemon juice is refreshing.

- Change the fluids often to keep them fresh.
- Use ice chips or popsicles as excellent ways to give fluids. Also, they help to keep the mouth moist and feeling fresh.
- Raise the person's head when helping to drink. Use a couple of pillows or gently support the base of the head with your hand. It is almost impossible to drink when lying flat.
- Ask the person to take small sips and not big gulps to help prevent choking.
- Use a straw that bends when the person is strong enough to suck on it.
- Try a spill-free thermos bottle or cup to make drinking easier if lips no longer fit tightly around the rim of a glass.
- Use a gentle reminder if the person forgets to swallow. Sometimes softly stroking the side of the throat will help to stimulate swallowing.
- Offer fluids that are somewhat thick and easier to swallow such as milkshakes and applesauce, if the person has difficulty swallowing.

